



PROCEDURE

Post Exposure Prophylaxis (PEP) HIV Pre-Packed Starter Packs

Scope

Site	Service/Department/Unit	Disciplines
SCGH and OPH (SCGOPHCG)	Emergency Department, under the direction of Immunology	Medical, Nursing, Allied Health

NOTE: Pre-pack supply is applicable only outside of normal business hours (16:00hrs – 07:30hrs)

Introduction

This procedure provides guidance to all Sir Charles Gairdner Hospital (SCGH) staff involved in the prescription, administration, supply and management of patients receiving Post Exposure Prophylaxis (PEP) HIV Pre-Packed Starter Packs. This document supports and complies with related legislation, policy and other related documents [here](#).

1.0 General Information

Patients may present directly to the Emergency Department (ED) during or after business hours for the commencement of Human Immunodeficiency Virus (HIV) Post-Exposure Prophylaxis (PEP) treatment;

1. Following a potential occupational exposure to HIV (e.g. needlestick); OR
2. Presentation to ED following a potential exposure to HIV.

Post Exposure Prophylaxis (PEP) is a course of anti-HIV drugs that is to be taken as soon as possible, up to 72 hours after potential exposure to HIV infection, with the aim of reducing the risk of HIV transmission.

Once commenced, treatment is continued for 4 weeks unless HIV infection can be reliably excluded in the potential source.

PEP should generally not be prescribed after 72 hours but may be considered on a case-by-case basis in consultation with an Immunologist.

The management principle for the Emergency Department is to provide timely access to treatment (ie 7-day 'starter pack'), along with relevant baseline tests and referral for subsequent follow-up to either Sexual Health at RPH or Clinical Immunology at SCGH dependent on route of exposure

Consult with on-call Immunology via switch dial 91

2.0 Presentation

The different drug regimens are pre-packed into a clear plastic box with the paperwork and instructions included. See Dose section for regimens.

NOTE: There are TWO different PEP drug regimens: 2-drug and 3-drug regimen.

The regimen selected will depend on several factors including the clinical scenario, form of exposure, potential for side effects and expected compliance.

3.0 Indication

1. For [Post Exposure Prophylaxis \(PEP\) after Non-Occupational or Occupational exposure to HIV](#) as per the Australian National Guidelines
AND
2. **PEP use must be under the direction of Clinical Immunology** –contact the [on-call](#) Immunology registrar in and out of hours

IMPORTANT

The starter pack/s provided by the Emergency Department will provide treatment for **SEVEN** days only.

A referral for a follow-up appointment with a specialist PEP provider is required to ensure completion of a full 28-day course.

4.0 Precautions

Caution and consideration of more frequent renal monitoring if co-administered with other potentially nephrotoxic agents, although a short PEP course is unlikely to cause significant issues.^{ASHM}

5.0 Drug Interactions

- There are a number of important drug interactions with tenofovir, emtricitabine, and dolutegravir, consult [SCGOPHCG approved references](#) (e.g. Australian Medicines Handbook) for more information.
- Tenofovir disoproxil:
 - can cause nephrotoxicity; administration with other nephrotoxic drugs* may increase this risk; avoid combinations or monitor renal function closely (manufacturer contraindicates combination with adefovir).
- Emtricitabine
 - NRTIs can cause pancreatitis; administration with other drugs* that can cause this may increase this risk; avoid use if possible or monitor very closely.
 - They also cause peripheral neuropathy; administration with other drugs that also cause this may increase the risk.
- Dolutegravir
 - Dolutegravir is mainly metabolised by glucuronidation, and to a small extent by CYP3A4. It is also a substrate of some transporter proteins, including P-gp. Giving dolutegravir with drugs that induce these enzymes or transporters may decrease its concentration and efficacy. Conversely, giving dolutegravir with drugs that inhibit them may increase its concentration and the risk of adverse effects.

- If giving with a CYP3A4 inducer, double the recommended dolutegravir dose by giving it twice daily and monitor clinically (eg HIV viral load, dolutegravir concentration). Avoid combination with dolutegravir if HIV is resistant to other integrase inhibitors.

6.0 Prescribing Requirements

Prescribers responsibilities	If treatment is indicated per the Consultant Immunologist, ensure the correct regimen is selected and prescribed. Complete the paperwork and label/s: • Complete the A4 pre-filled prescription • Leave completed prescription and used plastic box in the green PEP returns imprest bin • Complete the dispensing label on the starter medicine box/s (see below for instructions) • Provide patient counselling. The following will be available in the imprest tubs to assist in counselling given the toxicities and interaction relating to this group of drugs: a. MILs (medication information leaflet) b. CMI (Consumer Medication Information)
Nursing responsibilities	• Completed prescriptions are placed in plastic sleeve attached to imprest maxi-bins • Communication with ward pharmacist when stock is running low (1 box left), to organise replacement
Pharmacy imprest assistant responsibilities	• Collect completed prescriptions/boxes from return bin during imprest days • Notify the pharmacist if the number of scripts does not match the number of starter packs used • File prescriptions in designated file in stores • Replace pre-packs accordingly.
Clinical Pharmacist responsibilities	• Provide education to ED staff regarding procedure in and out of hours • Audit process regularly to assess compliance • Troubleshoot replacement stock if running low before imprest days • Review prescriptions in stores for compliance over audit period • Once reviewed; ○ photocopy for auditing purposes during audit period ○ send to medical records for filing in patient medical notes

6.1 Prescriptions - See [Appendix 2](#)

6.2 Special Supply Requirements

Supply of medicine via pre-packs (not dispensed via a pharmacy) is usually avoided due to complicating issues relating to legal requirements, traceability and increased risk.

In this circumstance, timely access to antiretroviral therapy ('starter pack') is critical and would otherwise result in a pharmacist call-in after hours.

Due to the above reasons and the predicted small number of times this may occur, pre-packed starter packs for the 2-drug and 3-drug regimen will be available on imprest, supplied in plastic clear boxes containing the relevant regimen, this guideline, an A4 pre-

filled script and a reformatted label/s to increase adherence to legal requirements and decrease risk. This will be audited to assess compliance.

1. Supply During working hours 07:30hrs – 16:00hrs
 - Contact the ED clinical pharmacist on #76731 for supply (do NOT use pre-packed starter packs)
 - Supply via a regular script, to be dispensed as a discharge medicine via Inpatient Pharmacy.
2. Supply Outside of normal business hours 16:00hrs – 07:30hrs
 - Supply pre-packed PEP as per this outlined procedure

7.0 Dose

7.1 2-Drug Regimen Box

- **Tenofovir with emtricitabine 300mg / 200mg tablet (combination) x 7 tablets**
- Pre-labelled box must be completed by the prescriber (see [Appendix 1](#)) and returned to pharmacy
- Prefilled paper script for tenofovir disoproxil with emtricitabine 300mg / 200mg must be completed (see [Appendix 2](#))
- Documents to assist counselling: MILs (medication information leaflet) and CMI (Consumer Medication Information)
- *Imprest quantity: 4 pre-packed boxes*

7.2 3-Drug Regimen Box

- **Tenofovir with emtricitabine 300mg / 200mg tablet (combination) x 7 tablets PLUS**
- **Dolutegravir 50mg tablets x 7 tablets**
- the 3-drug regimen is the provision of both starter packs of medicine
- both packs are included in the large clear plastic box labelled 3-DRUG REGIMEN
- Pre-labelled starter packs of tenofovir with emtricitabine 300mg / 200mg AND dolutegravir 50mg must be completed by the prescriber (see [Appendix 1](#)) and returned to pharmacy
- The pre-filled paper script for BOTH tenofovir / emtricitabine AND dolutegravir must be completed (see [Appendix 2](#)) and returned to pharmacy
- Documents to assist counselling: MILs (medication information leaflet) and CMI (Consumer Medication Information)
- *Imprest Quantity: 2 pre-packed boxes*

8.0 Pregnancy and Breastfeeding

- Contact the Obstetric Medicines Information Service, phone: 6458 2723
- For out of hours queries contact the KEMH on call pharmacist via switch

Related Documents

Legislation:

- [Medicines and Poisons Act 2014](#)
- [Medicines and Poisons Regulations 2016](#)
- [Health Services Act 2016](#)

Authorising policy and standards

- [SCGOPHCG Medicines Management Policy and Procedure](#)
- [SCGOPHCG High Risk Medication Policy](#)
- [Mandatory Policy MP0077/18 Statewide Medicines Formulary Policy](#)

- [OD 0647/16 National Standard for User-Applied Labelling of Injectable Medicines, Fluids and Lines 2016](#)
- [Australian Commission on Safety and Quality in Healthcare. Recommendation for terminology, abbreviations and symbols used in medicines documentation, December 2016](#)
- NSQHS Standards: 1,4,5,6,8

Supporting documents

- National guidelines for post-exposure prophylaxis after non-occupational and occupational exposure to HIV. 2nd edition, ASHM 2016
<http://www.pep.guidelines.org.au/index.php>
- OD 0013/07 "Guideline For Non-Occupational Post-Exposure Prophylaxis (NPEP) To Prevent HIV In Western Australia
- RPH PEP guideline

Authorisation & Version Control

Policy Sponsor	Director of Clinical Services				
Policy Contact	Drugs and Therapeutics Committee Executive Officer (Senior Pharmacist)				
Date First Issued:	01/02/2018	Last Reviewed:	11/08/23	Review Date:	11/08/2024
Version No.	3				
Approved by:	SCGOPHCG Drugs and Therapeutics Committee			Date:	30/08/2023
Acknowledgments:	V2 acknowledgements: Head of Department Immunology, SCGH Pharmacy Department, Consultant Emergency Department, CNS Emergency Department. V3 acknowledgements: SCGH Pharmacy Department				
Standards Applicable	NSQHS Standards (Version 2): 				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix 1

SAMPLE LABEL ON PRE-PACK MEDICATION

Figure 1. Tenofovir/emtricitabine pre-pack label for completion by prescriber

KEEP OUT OF REACH OF CHILDREN

Tenofovir disoproxil 300mg/emtricitabine 200mg Tablets (pp7)
(Mylan)

Take ONE tablet in the morning (starter pack)

Patient Name: ***1 Date: ***2 / /

Batch No: Expiry:

Doctor: ***3

Prescription Reference No: ***4

10A

Sir Charles Gairdner Hospital Pharmacy
Hospital Ave, Nedlands WA 6009

Figure 2. Dolutegravir Pre-pack label for completion by prescriber

KEEP OUT OF REACH OF CHILDREN

Dolutegravir 50mg Tablets (pp7) (Tivicay)

Take ONE tablet in the morning (starter pack)

Patient Name: ***1 Date: ***2 / /

Batch No: Expiry:

Doctor: ***3

Prescription Reference No: ***4

10A

Sir Charles Gairdner Hospital Pharmacy
Hospital Ave, Nedlands WA 6009

***Must be completed by prescriber at supply:

1. Patient name
2. Date of supply
3. Prescribers name
4. Write the prescription reference number

Note: Prescription Reference No. is Doctor's Initials, date of supply plus the number of prescription written for the day (IYYMMDDN) e.g. TM17122101

Appendix 2

2 (TWO) DRUG REGIMEN

Tenofovir disoproxil 300mg + Emtricitabine 200mg

Pre-packed Prescription Emergency Department

Attach ADR Sticker

ALLERGIES & ADVERSE REACTIONS (ADR)

☐ Nil known ☐ Unknown (tick appropriate box or complete details below)

Drug (or other)	Reaction/Date	Initials

Sign Print Date

AFFIX PATIENT IDENTIFICATION LABEL HERE

UR No:

Family Name:

Given Names:

Address:

DOB

Sex ☐ M ☐ FNOT A VALID
PRESCRIPTION UNLESS
IDENTIFIERS PRESENT

1st Prescriber to Print Patient

Name and Check Label Correct:

Formulary indication: For use as per the Australian National Guidelines, under the direction of a Consultant Immunologist. (Non PBS)

Prescriber to
initial: _____

Drug name and form	Strength	Dose route and frequency	Quantity
Tenofovir disoproxil + emtricitabine Tablets	300mg + 200mg	Take ONE tablet in the morning (starter pack)	7 tablets

Medication Label requirements on pre-packed box – See Pre-Packed labels	Prescriber to ☒ & initial
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***Must be completed by prescriber at supply:

1. Patient name
2. Date of supply
3. Prescribers name
4. Write the prescription reference number
 - a. Note: Prescription Reference No. is the Doctor's Initials, date of supply plus the number of prescription written for the day (Initials.YYMMDD.#day) e.g. TM1712211

☐
☐
☐
☐Label
completed

Initial: _____

Prescriber Details

Name:	Prescriber No.:
Signature:	Date: / /

Prescription Order

Tenofovir disoproxil/emtricitabine pre-pack replacement prescription is intended to cover each ED supply at Sir Charles Gairdner Hospital.

Delivery / Collection

Completed prescriptions will be collected from the impres PEP return bin and replaced on imprest days.

Contact Details

ED Pharmacist
Pager: 4604 or DECT 76731
Business hours: Mon-Fri (07:30-16:00)

Completed Prescriptions:

Leave original, completed form/prescription in the imprest maxi-bin

Pharmacy Use Only

2 drug regimen
replaced
PP7
#stock replaced
by:
date:

3 (THREE) DRUG REGIMEN

Tenofovir disoproxil 300mg + Emtricitabine 200mg PLUS Dolutegravir 50mg

Pre-packed Prescription Emergency Department

Attach ADR Sticker

ALLERGIES & ADVERSE REACTIONS (ADR)		
☐ Nil known ☐ Unknown (tick appropriate box or complete details below)		
Drug (or other)	Reaction/Date	Initials
Sign	Print	Date

AFFIX PATIENT IDENTIFICATION LABEL HERE

UR No:

Family Name:

Given Names:

Address:

DOB

Sex ☐ M ☐ F

NOT A VALID
PRESCRIPTION UNLESS
IDENTIFIERS PRESENT

1st Prescriber to Print Patient Name and Check Label Correct:

Formulary indication: For use as per the Australian National Guidelines, under the direction of a Consultant Immunologist. (Non PBS)

Prescriber to initial: _____

Drug name and form	Strength	Dose route and frequency	Quantity
Tenofovir disoproxil + emtricitabine Tablets	300mg + 200mg	Take ONE tablet in the morning (starter pack)	7 tablets

PLUS

Drug name and form	Strength	Dose route and frequency	Quantity
Dolutegravir Tablets	50mg	Take ONE tablet in the morning (starter pack)	7 tablets

Medication Label requirements on BOTH boxes – See Pre-Packed labels	Prescriber to ☒ & initial
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***Must be completed by prescriber at supply:

1. Patient name
2. Date of supply
3. Prescribers name
4. Write the prescription reference number
 - a. Note: Prescription Reference No. is the Doctor's Initials, date of supply plus the number of prescription written for the day (Initials.YYMMDD.#day) e.g. TM1712211

☐
☐
☐
☐
Label completed on BOTH boxes
 Initial: _____

Prescriber Details

Name:	Prescriber No.:
Signature:	Date: / /

Prescription Order

Tenofovir disoproxil/emtricitabine PLUS Dolutegravir pre-pack replacement prescription is intended to cover each ED supply at Sir Charles Gairdner Hospital.

Completed Prescriptions:

Leave original, completed form/ prescription in the imprest maxi-bin

Delivery / Collection

Completed prescriptions will be collected from the imprest PEP return bin and replaced on imprest days.

Pharmacy Use Only

3 drug regimen replaced PP7 #	stock replaced by: date:
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Contact Details

ED Pharmacist
Pager: 4604 DECT 76731
Business hours: Mon-Fri (07:30-16:00)